Which treatment this guide covers National Cancer Control Programme (NCCP) 00219a is first-line EGFR-mutated advanced non-small-cell lung cancer (NSCLC); 00219b is switch-maintenance after stable disease following first-line chemotherapy; 00219c is after failure of at least one previous chemotherapy regimen. Global NCCP eligibility requires a validated activating EGFR mutation in each setting. These are advanced-disease settings, not adjuvant treatment.

NCCP 00219 · VERSION 6

Erlotinib monotherapy

The usual dose is 150 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any change is agreed with your team.

Daily EGFR-targeted tablets · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Lungs

erlotinib

New cough or breathlessness needs urgent assessment.

2 Bowel

erlotinib

Diarrhoea can cause dehydration; follow the symptom plan.

3 Skin and nails

erlotinib

Report rash, painful nails or peeling skin.

4 Eyes

erlotinib

Eye pain or blurred vision needs prompt review.

5 Liver

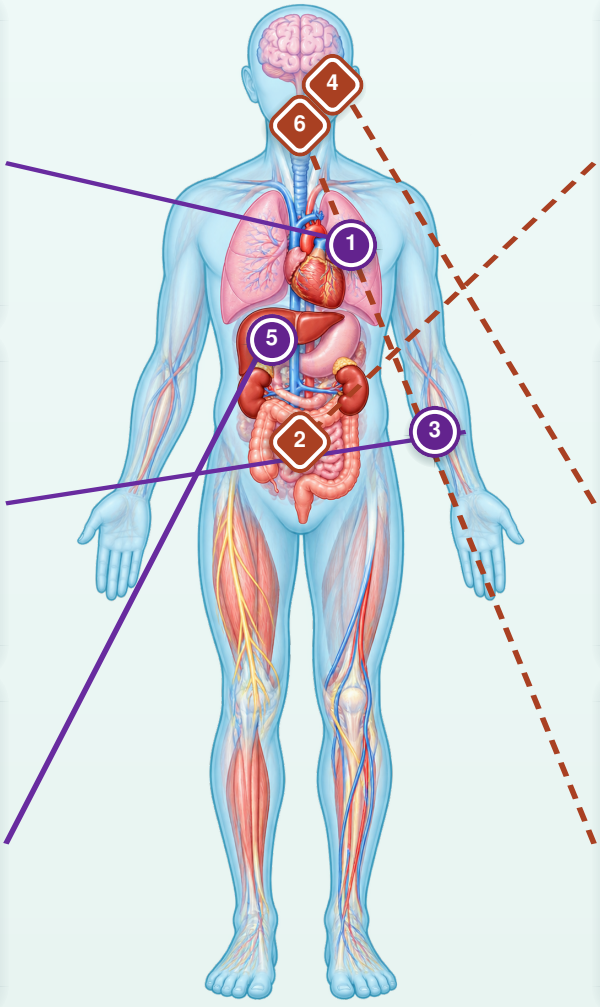
erlotinib

Blood tests check for liver injury.

6 Mouth

erlotinib

Report soreness or difficulty eating.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or persistent abdominal pain; blistering or peeling skin; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit's temperature and 24-hour contact instructions. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

The usual dose is 150 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any change is agreed with your team.

Take at least 1 hour before or 2 hours after food. If a dose is missed, do not replace it; take the usual next dose. If you vomit within a few hours after taking it, do not repeat the dose. Ask pharmacy before changing smoking habits or adding medicines for stomach acid.

Checks and follow-up

NCCP specifies blood count, kidney and liver tests every 60 days. Diarrhoea or dehydration needs extra kidney and electrolyte tests, including potassium. Liver problems or interacting medicines can require closer tests. Scans follow the Consultant's plan.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

Diarrhoea and fluids

Keep the prescribed anti-diarrhoea plan available from the first dose. Report loose stools early and keep drinking if able. Persistent or severe diarrhoea, weakness, inability to drink or much less urine requires urgent advice. The team decides whether to hold treatment and when it is safe to restart.

Skin, nails and mouth

Use the agreed skin, nail and mouth-care plan. Report rash, painful nails or mouth sores early. Emollients and sun protection may help; the team decides whether creams, antibiotics or other treatment are needed. Blistering or peeling skin needs urgent assessment.

Nausea medicines

Erlotinib has minimal-to-low oral emetic risk. Agree any as-needed anti-sickness medicine with the team. Routine infusion premedication is not required.

Medicine interactions

Give pharmacy all medicines and supplements. Strong CYP3A4 inducers or inhibitors, grapefruit and medicines for stomach acid can change exposure. Avoid proton pump inhibitor combinations unless the specialist resolves the plan. Necessary antacids require source-specific separation; ask for written timing. Warfarin needs extra blood-clotting checks. Smoking reduces exposure; get cessation support and do not change your cancer dose yourself.

Pregnancy, fertility and breastfeeding

Avoid pregnancy. Use effective contraception during treatment and for at least 2 weeks after the final dose. Do not breastfeed during treatment or for at least 2 weeks after the final dose. Discuss fertility and future pregnancy plans before starting.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital's urgent-care plan. Emergency: 112 / 999.



Your complete treatment reference

https://sactcheck.com/patient/00219/

Public information only. No personal information is encoded.